

Name: Lisa A English | DOB: 7/25/1964 | MRN: 055656 | PCP: Jung Wook Kang Smith, MD | Legal
Name: Lisa A English

Progress Notes

Signed Jun 22, 2026

Progress Notes by Ahmad Malik at 06/22/26 1406

Deaconess Care Group Hospital Progress Note

Patient Name: Lisa A English
MRN: 055656
DOB: 7/25/1964
Date of Admission: 6/19/2026
Date of Service: 6/22/2026
PMD: Kang Smith, Jung Wook

Chief complaint: Debility

History

The patient is a 61 y.o. female with a past medical history notable for chronic debility/wheelchair-bound, former smoker/severe COPD oxygen dependent on chronic steroids and dupixent, chronic respiratory failure with hypoxia and hypercapnia, hypertension, hyperlipidemia, type 2 diabetes mellitus, CAD, depression/anxiety/PTSD, chronic anemia, kidney stones, GERD.

She was discharged home yesterday from hospital due to severe COPD exacerbation, short words Augmentin for possible right upper extremity cellulitis.

She presented back from home via EMS due to an inability to care for herself, she requires help at home in transition between recliner to the scooter, her son is not able to help her, she has been sitting in the recliner since discharge yesterday from hospital.

Afebrile, heart rate 180, respiratory 20, BP 152/69, 97% on 2 L
Lab significant for WBC 11.5, hemoglobin 9.3, glucose 109, CPK 22, ABG pH 7.4/62.8

She received DuoNeb, admission requested for potential placement.

Medically ready. Pending placement

Consultants:
None

Procedures:

None

Anti-infectives:

None

Subjective: Patient was seen and evaluated at the bedside. Denies new concerns. Reporting anxiety requesting IV morphine

Objective:**Vitals:**

	06/22/26 0803	06/22/26 0900	06/22/26 1030	06/22/26 1031
BP:				
Pulse:		80		
Resp:				
Temp:				
TempSrc:				
SpO2:	94%		95%	95%
Weight:				
Height:				

Physical Exam:

General: Awake, alert and oriented, **anxious**

Cardiac: S1-S2 regular rate and rhythm with no murmurs, gallops or rubs

Respiratory: Clear to auscultation bilaterally with no wheezes, rhonchi or crackles

Abdomen: Soft, nontender, nondistended with no organomegaly

Extremities: No edema

Neuro: No focal deficits

Psych: Affect normal, speech fluid and normal

Labs Reviewed**Recent Results (from the past 24 hours)****POCT GLUCOSE**

Collection Time: 06/21/26 5:54 PM

Result	Value	Ref Range
POC Glucose	181 (H)	70 - 99 MG/DL

POCT GLUCOSE

Collection Time: 06/21/26 9:38 PM

Result	Value	Ref Range
POC Glucose	150 (H)	70 - 99 MG/DL

POCT GLUCOSE

Collection Time: 06/22/26 9:36 AM

Result	Value	Ref Range
POC Glucose	170 (H)	70 - 99 MG/DL

Medications Reviewed:

• aspirin	162 mg	Oral	Daily
• budesonide	500 mcg	Nebulization	BID
• busPIRone	5 mg	Oral	BID
• docusate	100 mg	Oral	BID
• enoxaparin	40 mg	Subcutaneous	Q24H
• FLUoxetine	20 mg	Oral	Daily
• furosemide	20 mg	Oral	Daily
• guaiFENesin	400 mg	Oral	qid (q4h while awake)
• insulin lispro (HumaLOG, AdmeLOG) vial	0-6 Units	Subcutaneous	TID AC
• ipratropium-albuterol	3 mL	Nebulization	6 times per day
• lisinopril	20 mg	Oral	Daily
• metoprolol succinate	50 mg	Oral	BID
• pantoprazole	40 mg	Oral	Daily
• predniSONE	20 mg	Oral	Daily
• sulfamethoxazole-trimethoprim	1 tablet	Oral	Once per day on Monday Wednesday Friday
• theophylline	300 mg	Oral	Daily

Assessment/Plan:**Morbid obesity due to excess calories (HCC)****Physical debility****DM hyperosmolarity type II, uncontrolled (HCC)****Chronic respiratory failure with hypoxia and hypercapnia****Generalized anxiety disorder****Dependence on non-invasive ventilation****COPD, end-stage****Plan:**

PRN nebs q4h, p.o. prednisone at home dose

Mucinex, Tessalon p.r.n. for cough

Hold metformin, ISS

Home O2 2-3 L nasal can baseline, BiPAP q.h.s.

PT/OT, fall precautions

Consult case management for potential placement

DVT prophylaxis: Lovenox, SCD

Code status: Full code

Disclaimer: Beginning in the spring of 2021, MyChart will allow progress notes to be visible to patients. Please note that these notes will include professional medical terminology that may be somewhat confusing without some interpretation from your medical professional team. The intent of progress notes is to communicate information between any medical professionals involved in your case, or to serve as future reference for myself or any other provider when reviewing your case, as well as a reference for the patient viewing the record. Please ask a member of the medical team if you have any questions about terminology or content of note.

This has been electronically signed by:



Ahmad T Malik, MD
Deaconess Care Group
Hospitalist
6/22/2026